populations. Potential influences of ethnicity, age, sex, patient adiposity, colonoscope lens and processor quality, quality of bowel preparation, and polyp location are commonly not considered in discussions of optimal benchmarks for CSSP-DR. We hereby share the findings from our recently published study,[3] which raise interesting questions for consideration in future studies.

In our study, we observed significant sex disparities in CSSP-DRs in a general outpatient colonoscopy population (n = 2097). When male and female individuals were compared, CSSP detection was 15% and 20%, respectively ($P <$].05), and sessile serrated lesions (SSLs) were 11% and 17%, respectively ($P < .05$). CSSPs and SSLs were independently associated with female sex and synchronous adenomas and SSLs also with body mass index $\geq$25 kg/m^2. SSL and adenoma detection were highly correlated ($r = 0.83$, $P = .002$). Therefore, whether CRC develops through a serrated or an adenoma pathway, CSSP-DR may provide a window to overall quality of colonoscopy. The question we pose is whether male and female individuals have inherently different susceptibilities to CSSPs. Studies of screening populations aged $\geq$50 years or with positive fecal immunologic test results have a bias toward greater proportions of males with adenomas.[4] Furthermore, the fecal immunologic test is not particularly useful for CSSP detection.[5] There hence remains a scarcity of research regarding patient and procedure characteristics associated with CSSP detection. Inasmuch as the diagnostic criteria for serrated polyps have recently been revised,[6] future studies should examine these characteristics in more detail. It is possible that artificial intelligence in colonoscopy may have a role in defining CSSP-DR, including in populations non-screening colonoscopy.

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Ali Galalah Mostafa Ismail, MD
Royal Perth Hospital
Perth, Australia
Medical School
The University of Western Australia
Nedlands, Australia
Vidit Lall
Sir Charles Gairdner Hospital
Nedlands, Australia
Medical School
The University of Western Australia
Nedlands, Australia
Oyekoya Taiwo Ayonrinde, MBBS, PhD, FRACP
Medical School
The University of Western Australia
Nedlands, Australia
Department of Gastroenterology and Hepatology
Fiona Stanley Hospital
Murdoch, Australia
Faculty of Health Sciences
Curtin University
Bentley, Australia

REFERENCES

1. Anderson JC, Hisey W, Mackenzie TA, et al. Clinically significant serrated polyp detection rates and risk for post colonoscopy colorectal cancer: data from the New Hampshire colonoscopy registry. Gastrointest Endosc 2022;96:310-7.
2. Macaron C, Rouphael C, Burke CA. Setting a benchmark for serrated polyp detection rate: defining the target and terminology comes first. Gastrointest Endosc 2022;96:318-20.
3. Lall V, Ismail AG, Ayonrinde OT. Disparate age and sex distribution of sessile serrated lesions and conventional adenomas in an outpatient colonoscopy population–implications for colorectal cancer screening? Int J Colorectal Dis 2022;37:1569-79.
4. Corley DA, Jensen CD, Marks AR, et al. Variation of adenoma prevalence by age, sex, race, and colon location in a large population: implications for screening and quality programs. Clin Gastroenterol Hepatol 2013;11:172-80.
5. Crockett SD, Nagtegaal ID. Terminology, molecular features, epidemiology, and management of serrated colorectal neoplasia. Gastroenterology 2019;157:949-66.
6. WHO Classification of Tumours Editorial Board. Digestive system tumours: Who Classification of Tumours. 5th ed. World Health Organization, Philippines. p. 635.

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Response:

We thank Ismail et al[1] for their comments and for highlighting the importance of identifying risk factors associated with serrated polyps. Inasmuch as sessile serrated polyps can be difficult to distinguish from hyperplastic polyps,[2] we have proposed clinically significant serrated polyps (CSSPs) as a surrogate outcome for sessile serrated polyps (SSPs), which include any sessile serrated polyps, traditional serrated adenomas, large ($\geq$1 cm) hyperplastic polyps (HPs), or those proximal HPs >5 mm.[3] As published in a prior study,[4] an analysis of data from the New Hampshire Colonoscopy Registry population demonstrated that higher body mass index and also smoking were strongly associated with CSSP.[5] Although older age and male sex have been observed to be associated with conventional adenomas, these factors do not appear to be major factors associated with serrated polyps. It is possible that some confounding factors such as smoking, which has been strongly linked with serrated histologic features[5,6] and with BRAF mutations and the CpG island methylator phenotype,[7] may explain the differences observed for the prevalence of CSSPs in male and female patients in the authors' analyses. We agree with the authors that identifying risk factors would help in stratifying patients for screening. Potentially, those at higher risk for serrated or conventional adenomas could be screened with colonoscopy, whereas other low-risk patients could receive noninvasive testing. Finally, we also agree with the authors that attention to clinically significant serrated detection rates (CSSDRs) is important in maximizing quality. In our study, we observed that even among patients whose examinations were performed by endoscopists with adenoma detection rates $\geq$25, examinations performed by endoscopists with higher CSSDRs were associated with lower postcolonoscopy risk of colorectal.[8]

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Joseph C. Anderson, MD
Geisel School of Medicine at Dartmouth
Hanover, New Hampshire, USA
New Hampshire Colonoscopy Registry
Lebanon, New Hampshire, USA
Christina M. Robinson, MS
Department of Gastroenterology and Hepatology
Dartmouth-Hitchcock Medical Center
Lebanon, New Hampshire, USA
New Hampshire Colonoscopy Registry
Lebanon, New Hampshire, USA
Lynn F. Butterly, MD
Geisel School of Medicine at Dartmouth
Hanover, New Hampshire, USA
Department of Gastroenterology and Hepatology
Dartmouth-Hitchcock Medical Center
Lebanon, New Hampshire, USA
New Hampshire Colonoscopy Registry
Lebanon, New Hampshire, USA

REFERENCES

1. Ismail AGM, Lall V, Ayonrinde OT. Are sex-specific benchmarks for clinically significant serrated polyp detection warranted? Gastrointest Endosc 2023;97:379-80.
2. Anderson JC, Lisovsky M, Greene MA, et al. Factors associated with classification of hyperplastic polyps as sessile serrated adenomas/polyps on morphologic review. J Clin Gastroenterol 2018;52:524-9.
3. Anderson JC, Butterly LF, Weiss JE, et al. Providing data for serrated polyp detection rate benchmarks: an analysis of the New Hampshire Colonoscopy Registry. Gastrointest Endosc 2017;85:1188-94.
4. Anderson JC, Rangasamy P, Rustagi T, et al. Risk factors for sessile serrated adenomas. J Clin Gastroenterol 2011;45:694-9.
5. Anderson JC, Calderwood AH, Christensen BC, et al. Smoking and other risk factors in individuals with synchronous conventional high-risk adenomas and clinically significant serrated polyps. Am J Gastroenterol 2018;113:1828-35.
6. Anderson JC, Pleau DC, Rajan TV, et al. Increased frequency of serrated aberrant crypt foci among smokers. Am J Gastroenterol 2010;105:1648-54.
7. Samowitz WS, Albertsen H, Sweeney C, et al. Association of smoking, CpG island methylator phenotype, and V600E BRAF mutations in colon cancer. J Natl Cancer Inst 2006;98:1731-8.
8. Anderson JC, Hisey W, Mackenzie TA, et al. Clinically significant serrated polyp detection rates and risk for postcolonoscopy colorectal cancer: data from the New Hampshire Colonoscopy Registry. Gastrointest Endosc 2022;96:310-7.

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